

## P2 Ophtho/ENT/Psychiatry Practice Questions

## Week 16

### Questions

1. An 71-year-old woman walks into your family practice office, complaining of a headache that started recently and scalp tenderness when she combs her hair. On fundoscopy you notice a swollen optic disc and flame hemorrhages. What is the most likely immediate treatment?
  - A. Refer to ophthalmology intra-ocular injection of anti-VEGF antibodies.
  - B. Prescribe NSAIDs, provide reassurance and advise patient for better sleep hygiene
  - C. Start high dose steroids
  - D. Refer to ophthalmology for laser therapy
  - E. Refer to optometry for a full eye exam
2. A 2-year old boy is brought into your family practice clinic for the well baby visit. You noticed the following image when checking his eyes with the ophthalmoscope. What is the most appropriate next step?

- i. Make an urgent referral to ophthalmology
- ii. Start topical steroid eyedrops immediately
- iii. Ask parents for previous close-up shots of the eyes for comparison
- iv. Refer to optometry for a more extensive eye exam
- v. Provide parents with reassurance that this is a self limiting condition and their son will grow out of it
- vi. Try ophthalmoscope from an different angle and reassess



- A. i, ii, vi
  - B. i, iii
  - C. ii, iv, v
  - D. v, vi
3. Which of the following statement is TRUE for presbycusis?
    - A. It is the most common conductive hearing loss in elderly
    - B. The loss is most common at higher frequency
    - C. It's reversible with early treatment
    - D. Weber test will lateralize to the affected ear
  4. A 39-year old male complains of hearing loss. He is a construction worker and is exposed to loud noises daily. You suspect that he may have noise-induced hearing loss. Which of the following findings is the most consistent with this diagnosis?
    - A. An air-bone gap on audiometric testing
    - B. Bone conduction greater than air conduction bilaterally when performing a Rinne test
    - C. A characteristic 4000 Hz notch on an audiogram

- D. Symptoms of vertigo and aural fullness
5. A 36-year-old male presents to your family practice with a concern of dizziness. He says the episodes last for about 30 seconds, are typically triggered when he “moves too fast,” and when they occur it feels like the room is “spinning around him.” He denies any changes in hearing. He does not take any medications and his past medical history is unremarkable. Given the most likely diagnosis, what treatment would you suggest?
- A. Reassurance, vestibular rehabilitation exercises, no pharmacologic treatment
  - B. Reassurance, rest, and symptomatic treatment (e.g. gravol)
  - C. Smoking cessation, reduced-salt diet, diuretics, anti-nausea drugs
  - D. Reassurance, NSAIDs, beta-blockers, calcium-channel blockers, tranquilizers, 5-HT agonists
6. Which of the following is an example of a cause of death?
- A. Cardiac arrhythmia
  - B. Septic shock
  - C. Suicide
  - D. Myocardial infarction
- 7) A 25-year-old patient presents to your office with right-sided painless vision loss and a left RAPD. Upon examination, you find that their optic disc appears to be swollen. What is the most appropriate next step?
- A) Reassure the patient it will resolve on its own
  - B) Consult with neuro-ophthalmology for management of optic neuritis
  - C) Consult with neurology to investigate underlying Multiple Sclerosis
  - D) B and C
  - E) B only
  - F) None of the above
- 8) A 55-year-old patient presents to your office to follow up on an injury after a recent car accident. The patient tells you that they’ve been having trouble driving at night often seeing glares and haloes around lights. What is the most likely diagnosis?
- A) Cataracts
  - B) Glaucoma
  - C) Vitreous hemorrhage
  - D) None of the above

- 9) You are on your ER rotation and a 20-year-old patient presents with significant eye discharge that started in the last 10 hours. On history, the patient tells you she had sexual intercourse 2 days ago. She did not use any protection methods. What is the most likely diagnosis?
- A) Anterior uveitis
  - B) Viral conjunctivitis
  - C) Gonococcal conjunctivitis
  - D) None of the above
- 10) A patient presents to your office with red eye, worsening vision, ocular pain, and photophobia. On exam you find that their cornea looks hazy. The patient tells you that they recently replaced their glasses with contact lenses. What is the most appropriate next step?
- A) Refer the patient to ophthalmology immediately
  - B) Advise the patient to continue wearing their lenses
  - C) Refer the patient to an optometrist
  - D) Start the patient on steroid medication
- 11) A patient presents to your office with hearing loss that started 2 days ago. The patient also complains of fullness in their ears and feeling like the world around them is “spinning”. What is the most appropriate next step?
- A) Order an MRI to rule out acoustic neuroma
  - B) Refer the patient to ENT urgently
  - C) Watchful waiting
  - D) Both A and B
  - E) None of the above
- 12) Which of the following is associated with ototoxicity? Pick all that apply.
- A) Cisplatin
  - B) Loop Diuretics
  - C) Aminoglycosides
  - D) A and C only
  - E) A, B and C

13) Which of the following is characteristic of benign positional paroxysmal vertigo (BPPV)? Pick all that apply

- A) Brief episodes of vertigo
- B) Motion sensitivity
- C) Vision loss
- D) A and B
- E) A, B and C

14) What is the best course of management for a patient presenting with the symptoms in Question 13?

- A) Referral to ENT
- B) Labyrinthectomy
- C) Particle repositioning
- D) None of the above

15) Which of the following is a red flag for headaches? Pick all that apply

- A) Worse when standing up
- B) Age over 60
- C) Associated with systemic symptoms (e.g. fever, chills)
- D) Chronic or insidious onset

16) Which of the following is an example of a migraine aura? Pick all that apply

- A) Coma
- B) Transient loss of vision
- C) Numbness and tingling
- D) Fever

17) In the event of patient death, which of the following is an indication to contact the coroner?

- A) Maternal deaths
- B) Family concerns about patient care
- C) Cancer deaths
- D) A and B only
- E) A, B and C

18) Which of the following is true about MAID?

- A) Patients under 18 years old are eligible for MAID under the present law in Canada
- B) A patient needs to give consent to MAID once during the process
- C) Patient eligibility needs to be certified by 2 independent physicians or nurse practitioners
- D) MAID must occur in a hospital

19) Which of the following is associated with a white light reflex (leukocoria) in a child ?

- A) Cataract
- B) Retinoblastoma
- C) Retinopathy of prematurity
- D) All of the above

20) Which of the following is an indication for referral to a pediatric ophthalmologist? Pick all that apply

- A) Nystagmus
- B) Constant deviation
- C) Occasional deviation for a month
- D) A and B only
- E) A, B and C

#### Answers

1. **C.** High clinical suspicion of giant cell arteritis, immediate high dose steroid helps prevent vision loss.
2. **B.** The photo depicts leukocoria in the right eye and it's critical to rule out retinoblastoma, which warrants an urgent referral to ophthalmology. Topical steroids are better to be reserved for ophthalmologists to prescribe; past photos are helpful to get a sense of the disease progression; this is not physiological leukocoria, which typically seen when light source is 15° off axis.
3. **B.** Presbycusis is a form of sensorineural hearing loss and typically affects both ears. It's progressive and irreversible. Conductive hearing losses are typically lateralized to the affected ear.
4. **C.** Noise-induced hearing loss is sensorineural and both air-bone gap on audiometry and bone > air on Rinne test suggest conductive hearing loss. Hearing loss plus vertigo symptoms would suggest Meniere's Disease.
5. **A.** The stem suggests BPPV and it can be treated with particle reposition maneuvers, which are considered as a type of vestibular rehabilitation exercise. B describes symptomatic treatment for vestibular neuronitis. C describes treatment options for Meniere's Diseases. D describes treatment for migrainous vertigo.

6. **D.** Septic shock & cardiac arrhythmia are MECHANISMS of death, and suicide is a MANNER of death.
7. **D.** Optic neuritis is one of the symptoms of MS. For a patient of this age, a referral to neurology to rule out MS is indicated
8. **A.** These symptoms are consistent with cataracts
9. **C.** Given the patient's history and symptoms, this is consistent with a gonococcal conjunctivitis
10. **A.** These symptoms are consistent with contact lens associated keratitis. A referral to ophthalmology is indicated since this can lead to permanent vision loss
11. **D.** This is a case of sudden SNHL which is an otologic emergency requiring referral to ENT and an MRI to rule out acoustic neuroma
12. **E.** All of these agents are ototoxic
13. **D**
14. **C.** Particle repositioning is the first course of management for patients with BPPV
15. **B and C.** Headaches that are worse when supine rather than standing up are a red flag due to increased ICP. Acute onset rather than insidious onset headaches are also red flags
16. **B and C.**
17. **D.** A patient dying of cancer is not in and of itself an indication to contact the coroner
18. **C.**
19. **D.**
20. **D.** A referral to an ophthalmologist is indicated if occasional deviation lasts beyond 3-4 months

## Week 17

### Questions

1. Sort the following types of dysphagia (i, ii, iii, iv, v, vi) into their types (A, B):
  - a. Mechanical - issue with solids only
  - b. Motility - issue with solids and liquids
    - i. Schatzki ring (an intermittent dysphagia)
    - ii. Jackhammer esophagus (an intermittent dysphagia with chest pain and repetitive peristaltic contractions)
    - iii. Scleroderma (a progressive dysphagia with heartburn)
    - iv. Eosinophilic esophagitis (a dysphagia presenting with corrugate rings, vertical furrowing, white papules often in young males)
    - v. Diffuse spasms (a “corkscrew” esophagus on barium swallow)
    - vi. Achalasia (a “bird’s beak” on barium swallowing often with regurgitation of bland food)
2. Which of the following statements is TRUE?
  - a. Modified barium swallow is used (over barium swallow) for the diagnosis of esophageal dysphagia rather than for the diagnosis for oropharyngeal dysphagia.
  - b. Methods to treat Schatzki rings, esophageal webs and esophageal strictures include a bougie (e.g. maloney, savary with guide-wire) and endoscopic balloon.
  - c. A scleroderma crisis involves dysphagia (e.g. wet vocal quality, nasal regurgitation) triggered by antibiotics, fever, infections, or recent surgery and is a reason for referral to emergency & SLP, and initiation of prednisone or IVIG.
  - d. Manometry is used to diagnose mechanical dysphagia, rather than motility dysphagia.
  - e. The most common esophageal cancer in the developed world is squamous cell cancer found in the proximal esophagus.
3. Which of the following is FALSE?
  - a. NMDA receptors are ionotropic and they mediate the excitatory action of glutamate.
  - b. Dopamine and serotonin targets share common pathways like MAO (metabolism) inhibition and VMAT (vesicular transport) inhibition.
  - c. Since GABA is inhibitory, its receptor modulators (e.g. benzodiazepines and barbiturates) are used for sedation, anticonvulsant, and anesthetics.
  - d. Metabotropic serotonin receptors work by changing the polarization state of the membrane for fast onset of action.
4. Part 1. Which of the following is NOT a myth about suicide? (i.e. which of the following is true?)
  - a. Increased public education programs (e.g. school campaigns) about suicide and its stigma have been shown to reduce suicide rates across many studies.
  - b. Self-harm is indicative of intrusive suicidal ideation and is a warning sign that shouldn’t be missed.



- c. Suicide has minimal biological basis and is typically due to unbearable distress, avoidance of difficult life situations, or as a method of controlling own's life.
  - d. Men are more likely to use lethal means for suicide and thus, more likely to die by suicide.
- 5. Part 2. Which of the following is NOT a myth about suicide? (i.e. which of the following is true?)
  - a. Teenagers and college students are the most at risk for suicide.
  - b. Suicide cannot be prevented.
  - c. Barriers to bridges, safe firearm storage and other actions to reduce access to lethal methods of suicide don't work.
  - d. While evaluating for suicide, physicians should avoid probing suicidal patients about the details of their plan for suicide - else it would cause the patient to further mentally decompensate and act on the plan.
  - e. Good self-esteem, future goals, and sense of belonging are protective factors against suicide.
- 6. Which of the following situations warrants SSRI or bupropion (NDRI) to be used first line?
  - a. Acute depression for bipolar disorder
  - b. Prevention of any mood episode, depression, or mania for bipolar disorder
  - c. Acute mania for bipolar disorder
  - d. Acute safety or tolerability concerns for bipolar disorder
  - e. Maintenance treatment for bipolar disorder
  - f. Rapid cycling bipolar disorder ( $\geq 4$  mood disturbances in a year)
  - g. Bipolar disorder with mixed features.
  - h. None of the above.
- 7. Which of the following is a feature of bipolar depression rather than unipolar depression (choose 3):
  - a. Early onset of first depression ( $< 25$  y.o)
  - b. Long duration of episodes ( $> 6$  mo)
  - c. Psychotic features
  - d. Initial insomnia
  - e. Hyperphagia +/- increased weight
- 8. Match the psychotherapy (A, B, C, D, E) to its description (i, ii, iii, iv, v)
  - a. Psychodynamic Psychotherapy (PDT)
  - b. Dialectical Behavioural Therapy (DBT)
  - c. Cognitive Behavioural Therapy (CBT)
  - d. Behavioural Activation (BA)
  - e. Interpersonal Therapy (IPT)
    - i. Scheduling pleasurable activities into your weekly calendar
    - ii. Identifying and examining schemas, automatic thoughts/thought distortions, core beliefs with thought recording

- iii. A collaborative patient-therapist treatment focused on a patient's relationships with others, conflicts, griefs, and role transitions
- iv. A loose therapy that evaluates subconscious defenses, transference (patient sees the "blank" therapist as someone else) through free associations, observation, validation, clarification, confrontation, and encouragement to elaborate
- v. Developed specifically for borderline personality disorder, patients analyze chains of events, vulnerability factors, actions, cognition, feelings to change maladaptive behaviours to adaptive behaviours. Delivered individually, through group skill training, over the phone, or via therapist consultations.

9. In which cases are psychotherapy suitable/indicated (choose 2)?

- a. A patient with severe autism who is unable to form meaningful relationships.
- b. A schizophrenic patient in either prodrome, active psychosis, or remission and recovery.
- c. A patient with bipolar disorder (I) who is in acute depression.
- d. A patient who is significantly suffering from PTSD but has no capacity of insight, low tolerance for frustration, and resistant to therapists in general.

10. Which medical illness may co-occur with depression?

- a. Epilepsy
- b. Connective tissue disease like lupus
- c. Hepatic disease
- d. Diabetes
- e. All of the above AND more!! (did I mention: cancer, ESRD, cardiac disease, HIV)

11. Which of the following is false about depression?

- a. Patients with depression are at higher risk of developing diabetes and CAD.
- b. MAOI and TCA are recommended for cardiac patients with depression due to its relatively safe side-effects profile.
- c. Patients with medical illnesses may be chronically stressed, which results in a hyperactive immune system and sympathetic nervous system, and increased catecholamines, which ultimately results in depression.
- d. In patients with cancer, 61-82% will not have depression.

12. Match the drugs (ABCDEFGH) to the reuptake transporter it inhibits (i, ii, iii, iv), the latter of which can be used more than once.

- a. Fluoxetine and escitalopram (SSRI)
- b. Amphetamine
- c. Amitriptyline and clomipramine (TCA)
- d. Tiagabine (anticonvulsant)
- e. Duloxetine and venlafaxine (SNRI)
- f. Bupropion (NDRI)

- i. Serotonin transporter (SERT)
- ii. Norepinephrine transporter (NET)
- iii. Dopamine transporter (DATA)
- iv. GABA transporter (GAT1)

13. Which of the following statements about the multi-axial formulation is correct?
- a. Axis I refers to clinical disorders, including major mental disorders and learning disorders
  - b. Axis II refers to psychosocial and environmental factors contributing to the disorder
  - c. Axis III refers to personality disorders and intellectual disabilities
  - d. Axis IV describes an assessment of functioning
  - e. Axis V refers to acute medical conditions and physical disorders
14. Which of the following is not true about a mental disorder?
- a. It reflects dysfunction in a psychological, biological or development process underlying mental functioning
  - b. It causes clinically significant distress or impairment in functioning
  - c. It includes socially deviant behaviour
  - d. None of the above
15. In a psychiatric history, which of the following are the main goals of assessing previous psychiatric history?
- a. Establishing a longitudinal course of the illness
  - b. Determining severity of the disorder
  - c. Understanding predisposing, precipitating and protective factors
  - d. All of the above
16. What is the “funnel” strategy in the psychiatric interview?
- a. Asking about predisposing factors first, then precipitating factors, then protective factors
  - b. Asking open ended questions first in the HPI, then more close-ended questions
  - c. Asking about more difficult topics first and ending the interview on a lighter topic
  - d. Asking about lighter topics first and ending the interview on more difficult topics
17. You are interviewing Blanche, who tells you she missed her last appointment because she was following Lizzo around the country on her world tour. She tells you she had to go to every concert because Lizzo is in love with her and they are meant to be together. Which of the following is the best description of Blanche’s beliefs?
- a. Grandiose delusions
  - b. Delusions of reference

- c. Erotomanic delusions
  - d. Jealous delusions
18. Which of the following would not fall under the “speech” portion of the mental status exam?
- a. Tachylalia
  - b. Latency
  - c. Cluttered speech
  - d. Pressure of speech
19. Sophia has been referred to you by her family doctor due to complaints of low energy and difficulty concentrating as well as low mood and a loss of enthusiasm to continue with her marathon training, which she was previously excited about. As you talk to her, Sophia also tells you that even though she’s been training less, been eating and sleeping a lot more than usual and has gained weight in the past month. When you question her about her low mood, she tells you that she is planning on getting a puppy soon, which she is very excited about. Which of the following are you most likely to diagnose Sophia with?
- a. Binge eating disorder
  - b. Major depressive disorder with atypical features
  - c. Major depressive disorder with mixed features
  - d. Major depressive disorder with melancholic features
20. You are treating Phil, a 47-year-old man with a new diagnosis of Major Depressive Disorder. Which of the following are you least likely to discuss with him as a potential treatment option today?
- a. Electroconvulsive therapy
  - b. Light therapy
  - c. Cognitive behavioural therapy
  - d. SSRIs

21. You are meeting with Rose who was recently diagnosed with major depressive disorder and after 3 months of psychotherapy, would like to try an antidepressant. Rose recently got a new job that she worries about a lot especially at night when she is trying to fall asleep. Which of the following would be the best SSRI to try first?
- Citalopram (Celexa)
  - Sertraline (Zoloft)
  - Paroxetine (Paxil)
  - Fluoxetine (Prozac)
22. You are a clerk in a family medicine practice. Amira, who is new to your family medicine clinic, complains that her 20mg daily dose of fluoxetine is giving her trouble sleeping at night. You discontinue her fluoxetine and prescribe paroxetine 10mg daily. A week later, Amira tells you her anxiety and insomnia is worse and she's experiencing constant shaking with continuing depressive symptoms. You up Amira's paroxetine dose to 30mg daily to treat the symptoms.
- One month later, you are on your ER rotation and you see Amira in the ER complaining of extreme fatigue, tachycardia, diarrhea, tremor, hyperreflexia and a widespread extreme burning sensation. She seems confused. What treatment options are you considering?
- Discontinue paroxetine
  - IV fluoxetine
  - Benzodiazepines
  - Cyprohepatadine
  - Increase paroxetine dose
  - B, C, E
  - A, C, D
  - All of the above

## Answers

- A = i, iv; B = ii, iii, v, vi

2. B is correct. A is false because modified barium swallow (along with FEES) is used for oropharyngeal dysphagia. B describes myasthenic gravis crisis, not scleroderma crisis which is a renal issue. C is false. D - squamous cell cancer is most common worldwide but adenocarcinoma (is distal esophagus) is most common in developed world (e.g. its associated with GERD and Barrett's esophagus).
3. D
4. D. Increased educational campaigns have "not corresponded with a decline in suicide rates." Self-harm is separate from suicide. Suicide has biological basis (e.g. reduction in serotonin binding in the prefrontal cortex, low serum cholesterol, and abnormal corticotropin releasing hormone binding/level).
5. E. Women and men aged 45-65 have the most absolute numbers of death by suicide with older adult males being most at risk. Suicide can be prevented by coping & crisis plans and treating mental illnesses. When evaluating for suicide, it is important to get as much detail as possible and avoid assumptions because specific plans can be a risk factor.
6. H. For acute mania, first-line treatment would be atypical antipsychotics (i.e. quetiapine, risperidone, paliperidone, asenapine), third gen antipsychotics (i.e. aripiprazole), anticonvulsant (i.e. divalproex/valproic acid), and antimanic drugs (i.e. lithium). For acute depression and maintenance treatment, it would be quetiapine, lurasidone, lamotrigine, and lithium. SSRI and bupropion (NDRI) are second line drugs and should be used very cautiously in those with rapid cycling bipolar disorder or bipolar disorder with mixed features.
7. ACE
8. A - iv. B - v. C - ii. D - i. E - iii.
9. B,C. While a patient with schizophrenia do have poor "ego strength" (i.e. non-intact reality), in the Psychotic disorder module, it said that "there is strong evidence for CBT for prodrome, active psychosis (voice hearer group), remission and recovery)." The patient with severe autism may not be a good candidate as they are not able to form relationships outside of therapy and may have an inability to reflect, think abstractly/symbolically, and integrate interventions. The patient with PTSD similarly does not have a strong "ego strength" (i.e. tolerance for frustration) or "psychological mindedness" (i.e. capacity for insight).
10. E
11. B. SSRI are first line while MAOI and TCA may prolong QT intervals so are contra-indicated. D is true because the prevalence of depression in cancer patients is 18-39%.
12. A - i. B -iii. C - i, ii. D - iv. E - i, ii. F - ii, iii.
13. A
14. C - socially deviant behaviour is not considered a mental disorder
15. D: these are all goals of getting previous psychiatric history
16. B: when taking an HPI, it can be helpful to move from general to specific to close-ended questions
17. C: Erotomaniac delusions are the fixed false belief that someone is in love with the individual affected by the delusion.

18. D: Pressure of speech, often seen in mania, is related to thought disorder rather than speech.
19. B: Sophia meets the criteria for major depressive disorder (low energy, difficulty concentrating, low mood, anhedonia) as well as atypical feature criteria (mood reactivity (mood brightens in response to thought of getting a puppy), hyperphagia and hypersomnia).
20. A: ECT should be reserved for treatment-resistant depression
21. C: Paroxetine (Paxil), if dosed at night can help reduce Rose's anxiety and insomnia because of its sedating effects.
22. G: Amira has serotonin syndrome because the fluoxetine was not tapered before starting paroxetine and because her paroxetine dose was subsequently increased. Paroxetine needs to be discontinued. Benzodiazepines can be considered for sedation and cyproheptadine may be considered.

## Week 18

### Questions

1. Which of the following is not a manifestation of hyperactivity/impulsivity:
  - a. Blurts out answers before question is finished
  - b. Unable to stay seated
  - c. Fidgets
  - d. Careless mistakes
  - e. Difficulty playing quietly
2. T/F: ADHD resolves upon adulthood
3. T/F: The DSM-5 criteria for learning disorders require a test of IQ
4. Which of the following is not a risk factor for learning disabilities:
  - a. Genetics
  - b. Perinatal maternal EtOH consumption
  - c. Presence of lisp
  - d. Low birthweight
5. T/F: Articulation disorders increase with age
6. Which of the following is true about language disorders:
  - a. Language disorders are more common in girls
  - b. Bilingual children with language disorders will preferentially develop one language only
  - c. 90% of late talkers will catch up on their own
  - d. Language disorders are highly heritable
7. Which of the following is true about ASD:
  - a. It is a degenerative disorder
  - b. Anyone with a diagnosis of ASD should have genetic testing with a clinical microarray
  - c. Patients often have a reduced brain size compared to the general population
  - d. 75% of those with ASD have Intellectual Disability
8. Treatment for aggression in ASD include:
  - a. Fluoxetine
  - b. Benzodiazepine
  - c. Risperidone
  - d. Amitriptyline
9. Which of the following is a risk factor for disruptive behaviour disorders:
  - a. Chronic physical illness
  - b. Female sex
  - c. High birth weight
  - d. Overactivity of amygdala
10. Which of the following is not a presentation of conduct disorder?
  - a. Lack of empathy
  - b. Guilt and remorse
  - c. Flat affect



- d. Theft
11. Which of the following is not in the same cluster of personality disorders?
- a. Borderline Personality Disorder
  - b. Obsessive-Compulsive Personality Disorder
  - c. Histrionic Personality Disorder
  - d. Antisocial Personality Disorder
12. What is the most common type of personality disorder?
- a. Antisocial Personality Disorder
  - b. Narcissistic Personality Disorder
  - c. Borderline Personality Disorder
  - d. Schizotypal Personality Disorder
  - e. Paranoid Personality Disorder
13. Which of the following situations meet the criteria A for PTSD?
- a. Failing an Exam
  - b. Almost losing your life from house fire
  - c. Having a big fight with your best friend
  - d. Being suspended from school
14. What are the first line pharmacological therapies used for PTSD?
- a. Fluoxetine, Paroxetine, Sertraline, Venlafaxine
  - b. Amitryptiline, Aripiprazole, Bupropion, Topiramate
  - c. Olanzapine, Fluvoxamine, Mirtazapine, Fluvoxamine
  - d. Fluoxetine, Paroxetine, Sertraline, Citalopram
15. How long do you need to show symptoms of PTSD to be qualified for a diagnosis?
- a. 1 week
  - b. 6 months
  - c. 12 weeks
  - d. 3 months
16. Which of the following disorders are dissociation symptoms concomitantly found in? (Can choose more than 1)
- a. Borderline Personality Disorder
  - b. PTSD
  - c. Panic Disorder
  - d. Narcissistic Personality Disorder
  - e. Autism Spectrum Disorder
17. Which scale is used to evaluate severity of Dissociation Disorder?

- a. PHQ-9
  - b. YBOCS
  - c. MMSE
  - d. Dissociative Experiences Scale II (DES)
18. Average non-clinical population score on DES is...
- a.  $\geq 26$
  - b. 30
  - c. 11
  - d. 40
19. Which of the following are not one of the risk factors for developing anxiety disorders?
- a. Family history of anxiety
  - b. Personal history of anxiety or mood disorders (i.e., MDD, ADHD)
  - c. Childhood stressful life events/trauma
  - d. Chronic medical illness
  - e. Being male
20. Choose medical conditions that can present as anxiety or panic disorders
- a. Hyper/hypothyroidism
  - b. Caffeine
  - c. Anemia
  - d. Vitamin B12 deficiency
  - e. COPD
21. What is the median age for onset of OCD?
- a. 7
  - b. 15
  - c. 25
  - d. 19
22. T/F: the difference between worry and obsession is that obsession is almost exclusively experienced just as a thought.
23. Choose a combination of SSRI side effects with its appropriate treatments.
- a. Sexual side effects: use benzodiazepine
  - b. Restlessness: use bupropion
  - c. Sleep disturbance: use trazodone
  - d. Indigestion: use pepcid
24. Which one of the following are an appropriate example for each type of exposure therapies for anxiety Disorders?

- a. Situational - hyperventilating to provoke trembling/shaking for panic disorder treatment
- b. Interoceptive - going back to the location where you were mugged, to treat PTSD
- c. Imaginal - running back and forth to simulate symptoms of panic disorder
- d. Virtual - exposure to computer simulation of a war you've fought in, to treat PTSD

## Answers

- 1. D
- 2. F. ADHD persists into adulthood in about 60%
- 3. F, but LDAO does require one!
- 4. C
- 5. F
- 6. D
- 7. B
- 8. C
- 9. A
- 10. B
- 11. B
- 12. C
- 13. B
- 14. A
- 15. D
- 16. A,B,C
- 17. D
- 18. C
- 19. E
- 20. A,B,C,D,E
- 21. D (earlier onset in men)
- 22. False; worry is almost exclusively experienced as a thought while obsession is experienced as intrusive thoughts, images, or impulses.
- 23. C
- 24. D

## Week 19

### Questions

1. Which one of the following is a hallmark of refeeding syndrome?
  - A. Hypophosphatemia
  - B. Hypomagnesemia
  - C. Hyperkalemia
  - D. Diuresis
2. Which one of the following is false regarding eating disorders?
  - A. Eating disorders can involve behaviors in both food intake and output
  - B. Some eating disorders have comorbidities with intellectual disabilities
  - C. Anorexia nervosa is associated with heat intolerance and diarrhea
  - D. Heritability and family history are risk factors for many eating disorders
3. Your 7 year old patient was brought into your clinic by their father for having a limp. As you examine the patient, you notice multiple bruises along their legs. You ask the father about the bruises and he tells you it's none of your business and he will sue you if you report the findings to CAS. Should you report to CAS?
  - A. No, at least not yet because you will face prosecution if your suspicions were eventually proved wrong
  - B. No, you can not report child abuse against the parents/guardian's will
  - C. Yes, any suspected child abuse should be reported and you will be granted immunity from civil action as long as the report is not made "maliciously or without reasonable cause"
  - D. Yes, because you are 100% certain that this is a child abuse case and the father will not win if he sued you
4. For a child with Indigenous heritage in need of protection from child abuse, rank the placement of the child in order of priority:
  - A. Another Indigenous family
  - B. A member of the child's band or Indigenous community
  - C. A member of the child's extended family
5. As a third year medical student on your psychiatry rotation, you are asked to assess a new patient. The patient tells you that they discovered 2 years ago that their cat is actually a tiny alien in disguise. They also state that they can hear their cat communicating with their alien friends through a walkie talkie every night. They also report that there were weeks where they felt very energetic and invincible and then they will fall into a depressive phase. How would you differentiate between a potential bipolar disorder with psychosis versus schizoaffective disorder in this case?

- A. If psychotic symptoms are present outside the major mood episodes for more than 2 weeks, it is schizoaffective disorder
  - B. If psychotic symptoms are present outside the major mood episodes for more than 2 weeks, it is bipolar disorder with psychosis
  - C. If psychotic symptoms were present during the mania episode, it is schizoaffective disorder
  - D. If the psychotic symptoms were present during the depressive episode, it is bipolar disorder with psychosis
6. Which of the following patients with schizophrenia will have the better prognosis?
- A. A patient who's first psychotic episode happened at 15 years old and they use cannabis
  - B. A married patient who suddenly developed psychotic episodes at 55
  - C. A patient with dementia who gradually developed symptoms of psychosis over 2 years
  - D. A patient with only negative symptoms
7. 7. What is responsible for the negative and positive symptoms of psychosis (multiple correct answers)?
- A. Negative symptoms are the result of a lack of dopamine in the mesocortical pathway
  - B. Positive symptoms are a result of excess dopamine in the mesolimbic pathway
  - C. Negative symptoms are the result of a lack of dopamine in the mesolimbic pathway
  - D. Positive symptoms are the result of a excess dopamine in the mesocortical pathway
8. What are the differences between first and second generation antipsychotics (multiple correct answers)?
- A. Clozapine is more effective than haloperidol, but there is a risk of agranulocytosis with clozapine use
  - B. Haloperidol has higher risks of producing extrapyramidal symptoms and low blood pressure compared to olanzapine
  - C. Quetiapine has a higher risk of producing metabolic and cardiovascular side effects compared to first generation antipsychotics.
  - D. Clozapine has a higher potency compared to haloperidol
9. Which of the following would NOT be considered an important part of informed consent?
- A. The healthcare provider should explain the most common and serious side effects of the procedure
  - B. The healthcare provider must explain the consequences of no treatment
  - C. Substitute decision makers should base their choice of consent off the patient's prior wishes
  - D. The healthcare provider must provide information on every detail of the procedure being consented to

- E. The patient must be able to understand the information they are being provided and appreciate the consequences of their decision
10. Which of the following statements regarding involuntary admission for psychiatric assessment is FALSE?
- A. Form 1 is valid for 7 days and can detain a patient for up to 72 hours
  - B. A person must have a mental disorder and pose imminent danger to themselves or others to meet the criteria of Form 1
  - C. Form 1 can only be completed by a psychiatrist
  - D. Patients have the right to contest a Form
  - E. Form 3 must be completed by a different physician than the physician who completed Form 1
11. JB was once a highly successful businessman and stock broker. His business empire came crashing down for many reasons, including money laundering and securities fraud. While operating his brokerage house JB developed a substance abuse disorder and was abusing multiple substances, including cocaine, alcohol, and "Ludes." Which of the following statements would NOT meet criteria for his substance abuse disorder?
- A. JB required greater and greater amounts of substance to achieve the same high
  - B. JB used substances while operating motor vehicles
  - C. Between uses, JB experienced headaches, cravings, sweats, and chills, among other symptoms
  - D. JB's social and occupational activities were maintained while he was abusing substances
  - E. JB had only ever intended to try "Ludes" once but began using them on a regular basis
12. BR, a 22 year old male, is a new patient of yours. When you ask him about his alcohol consumption habits, he admits that he drinks more than he probably should. His AUDIT score flags him for hazardous drinking. He also explains that although he has not tried to reduce drinking, he occasionally feels fidgety, itchy, sweaty, and has a mild-moderate headache. Which of the following would NOT be a part of your next steps?
- A. Assess him for withdrawal symptoms using the Clinical Institute Withdrawal Assessment of Alcohol Scale (CIWA) and consider starting him on thiamine, a multivitamin, and folic acid
  - B. Advise him of his AUDIT screening result and provide advice to reduce or stop drinking
  - C. Determine his Stages of Change stage to help move him to the next step
  - D. Refer him to a concurrent disorders program
  - E. Explore his ambivalence towards reducing his drinking
  - F. Assess him for any underlying psychiatric comorbidities that may be causing or exacerbating his alcohol use
  - G. Ask him about changes in his mood, functioning, and dangerous activity associated with alcohol use

13. Which of the following statements regarding the assessment of concurrent disorders is FALSE?
- A. Urine toxicology can result in criminal charges if illegal substances are identified
  - B. Components of a good patient history include substance use, psychiatric history, and collateral information from other sources if necessary
  - C. Substance use disorders are the most common comorbid disorder for people with severe mental illness
  - D. Recovery involves empowering patients, improving their functioning, and engaging them with care teams
  - E. The effects of substances can often mimic psychiatric disorders
14. Which of the following statements regarding different concurrent psychiatric and substance abuse disorders is FALSE?
- A. Anxiety disorders often precede alcohol abuse disorder
  - B. Genetics, environmental factors, and psychological factors all influence the development of concurrent factors to varying degrees
  - C. Mood disorders nearly always precede the development of a substance use disorder
  - D. Tobacco is the most commonly abused substance in individuals with concurrent schizophrenia
  - E. Psychiatric disorders and substance abuse disorders are best treated simultaneously

## Answers

- 1. **A.**
- 2. **C.** Anorexia nervosa is associated with cold intolerance and constipation.
- 3. **C.**
- 4. **C, B, A.** According to the Child and Family Services Act.
- 5. **A.**
- 6. **B.** Positive prognostic factors of schizophrenia include: late onset, acute onset, good premorbid functioning, married, exclusively positive symptoms. Negative prognostic factors of schizophrenia include: early onset, insidious onset, poor premorbid functioning, single/divorced/widowed, negative symptoms.
- 7. **A, B.**
- 8. **A, B, C.**
- 9. **D.** When obtaining informed consent, healthcare providers must provide the patient with information that a reasonable person would want to know. This would not include explaining every detail of a procedure you are seeking consent to provide.
- 10. **C.** Form 1 (Application for Psychiatric Assessment) can be completed by any physician if they feel the criteria have been met.
- 11. **D.** JB meets many of the criteria for a substance use disorder. However, one criteria used for diagnosis of a substance use disorder is reduced or given up social, occupational, or recreational activities due to substance use. JB's ability to maintain his involvement in social and occupational

activities would not meet this criteria. However, JB still meets many other criteria for a substance use disorder, and a diagnosis of substance use disorder would be appropriate. Requiring increased amounts of substance to achieve the same high is tolerance. Substance use in situations where it is physically hazardous, like driving, is a criteria for substance use disorder. The symptoms described by JB are likely withdrawal symptoms. Substance use taken in larger amounts or over a longer period of time than intended is a criteria for substance use disorder.

12. **D.** Referral to a concurrent disorders program would not be indicated at this stage. We have not yet determined that BR has a concurrent psychiatric disorder, nor have we determined the severity of the psychiatric disorder and the severity of his alcohol abuse disorder. Assessing BR with the CIWA is a good place to start since he endorses alcohol withdrawal-like symptoms. If he meets criteria, starting him on thiamine, a multivitamin, and folic acid will be beneficial. He may also score high enough to warrant the use of lorazepam or diazepam. Advise him of his AUDIT score to indicate your level of concern, and encourage him to reduce or stop drinking. Explore whether he is in the pre-contemplation or contemplation Stage of Change, since he is aware he drinks more than he should but has not attempted to reduce his drinking; this will help you determine how to move him to the next Stage, whether that is contemplation or preparation. Exploring his ambivalence towards reducing his drinking is a part of Motivational Interviewing, and provides an opportunity to resolve his ambivalence and move him in the right direction, even if he does not have a diagnosis of alcohol abuse disorder. Taking a more detailed history will help you determine if there is a psychiatric disorder that is contributing to or feeding his alcohol abuse, and addressing both will be required. Finally, the first step that should be taken in all cases like this would be to take a more detailed history, with an additional focus on BR's alcohol-related behaviours in order to determine if he meets diagnostic criteria for a substance use disorder.
13. **A.** Patients should be aware that their care is free from criminalization. Urine toxicology is helpful (although not perfectly reliable) to help understand a patient's drug use. It can also help identify a substance used if the patient does not know what they took.
14. **C.** Mood disorders can lead to the development of a substance use disorder, but substance use disorders may also precede the development of a mood disorder.



## Week 20

### Question

1. Match the term to the correct description.

- A. N1 Sleep
- B. N2 Sleep
- C. N3 Sleep
- D. REM Sleep

1. Where vivid dreaming occurs and may be important for memory consolidation. EEG shows low-voltage fast waves, muscle atonia and rapid eye movements. Sawtooth wave form, theta activity and slow alpha activity.

2. Transitional role in sleep-stage cycle. EEG shows transitions from wakefulness (rhythmic alpha waves) to low-voltage, mixed frequency waves.

3. Related to memory consolidation. EEG shows low voltage fast activity characterized by the presence of sleep spindles and K-complexes.

4. Primary function is daily brain recovery and secretion of growth hormone is greatest during this stage. EEG shows high voltage, slow-wave activity, large delta waves.

2. What is commonly found on EEG for patients with fibromyalgia or chronic pain?
- a. Alpha wave intrusion in N2 Sleep
  - b. Sleep spindles and K complexes in N3 sleep
  - c. Large delta waves in REM sleep
  - d. Sawtooth waveforms in N1 sleep
3. What findings on EEG for depression? Select all that apply.
- a. Short REM latency
  - b. Decreased REM density
  - c. Reduced slow wave sleep
  - d. Less fragmentation
4. Which of the following are not clinical indications for sleep studies?
- a. Obstructive sleep apnea

- b. Narcolepsy
  - c. Parasomnia
  - d. Insomnia
5. M.R is a 23-year-old male who has come to your office complaining of excessive daytime sleepiness. His partner, who accompanies him on this visit, mentions that he has been more tired lately when he comes home from school and has trouble concentrating on tasks. They often find him has dozed off in front of the TV while waiting for dinner. He has restless sleep during the night, and they are getting tired of having their own sleep disrupted with his loud snores. He does little else but sleep during the weekend and his partner worries that might be a sign of depression. His physical exam is remarkable for blood pressure of 150/70, oxygen saturation of 95% on room air, body mass index of 35 kg/m<sup>2</sup>. Respiratory, cardiac, and abdominal examination are otherwise unremarkable. What is at the top of your differential diagnosis?
- a. Narcolepsy
  - b. Obstructive sleep apnea
  - c. Depression
  - d. Insomnia
6. You decide that patient M.R is at high risk for obstructive sleep apnea (OSA) and request an in-lab attended polysomnography. The Apnea/Hypopnea Index was 23.1. Does he have OSA and if so, what would the classification be?
- a. Normal
  - b. Mild
  - c. Moderate
  - d. Severe
7. You diagnose M.R with OSA. What are your possible treatment options? Select all that apply.
- a. Continuous positive airway pressure (CPAP)
  - b. Mandibular advancement device (MAD)
  - c. Uvulopalatopharyngoplasty
  - d. Bilateral positive airway pressure (BiPAP)
8. What is the first-line treatment for insomnia?
- a. Cognitive Behavioural Therapy
  - b. Sleep Hygiene
  - c. Cannabis
  - d. Benzodiazepines
9. D.P is a 27-year-old male who was recently diagnosed with REM behaviour disorder. A condition where dream enactment behaviours that merge during a loss of REM sleep atonia. He's concerned about the implications of his diagnosis and if he's at risk for any other diseases. What would you tell him?
- a. Increased risk of abnormal heart rhythms, heart attacks and heart failure.
  - b. Increased risk for insulin resistance
  - c. Increased risk of Parkinson's Disease, Multisystem Atrophy, or Lewy Body Dementia.
  - d. Increased risk of impotence and lack of sex drive

10. What is one of the main differences between Type I and Type II Narcolepsy?
  - a. Presence of cataplexy; Yes for Type I and No for Type II
  - b. Presence of sleep onset REM period (SOREMP)
  - c. Excessive daytime sleepiness
  - d. CSF Orexin <1000pg/ml in Type I and >1000 pg/ml in Type II
11. Which of the following are treatments for restless legs syndrome?
  - a. Calcium channel blockers
  - b. Beta blockers
  - c. Dopamine agonists
  - d. Iron supplementation
12. F.U. is a 21-year-old female, lawyer, married and mother of 3-year-old son. For the past 6 months, she has visited many different health care providers because she is concerned about having cancer. She currently has no symptoms. You preform a detailed history, physical exam and investigations, all of which were negative for cancer. You explain the results to the patient, but she refused to accept them and continued to insist that she had cancer. What is at the top of your differential diagnosis?
  - a. Schizophrenia
  - b. Factitious disorder
  - c. Conversion disorder
  - d. Illness anxiety disorder
13. 14. A.K. is a 20-year-old female reports with numbness and severe weakness on the left side of her body, specifically her leg. She describes being clipped by a vehicle crossing an intersection on a bike. She screened negative for a concussion and was subsequently sent home. Since the incident, 6 months ago, the patient reports her dizziness has converted to double vision, has difficulty swallowing like there is a lump in her throat, and notices occasional slurred words. Patient continues to have difficulty walking and loss of balance since the accident. Patient reports having increased stress and difficulty completing schoolwork in the past couple of months. History and physical examinations showed inconsistent findings that could not be explained by an organic or neurological cause. Investigations, including an MRI of the brain and spinal cord, lumbar puncture and bloodwork, were unremarkable. What is at the top of your differential diagnosis?
  - a. Guillain-bare system
  - b. Conversion Disorder
  - c. Myasthenia gravis
  - d. Multiple sclerosis
14. How do you differentiate between premenstrual dysmorphic disorder from premenstrual exacerbation of depression?
  - a. PMDD can have other symptoms besides sadness
  - b. Depression is not exacerbated during the premenstrual period usually
  - c. PMDD has minimal/absent symptoms in the weeks between menses
  - d. Depression has more severe and debilitating symptoms
15. Which antidepressant is associated with teratogenicity?

- a. Sertraline
  - b. Bupropion
  - c. Amitriptyline
  - d. Amoxapine
16. Match the syndrome with its behavioural phenotype:
- I. Hypervocalisation
  - II. Jerky movement
  - III. Neonatal hypotonia
  - IV. Anxiety and agitation
- a. Angleman syndrome
  - b. Williams syndrome
  - c. Prader-Willi syndrome
  - d. Fragile-X syndrome
17. Which of the following is NOT a classic symptom in a dual diagnosis of intellectual disability and behaviour problems/mental health problems:
- a. Aggressive behaviour
  - b. Difficulty with communication
  - c. Mood dysregulation
  - d. Failure to thrive
18. A family physician has been seeing a patient for several years and has built up a strong therapeutic relationship with them. During a visit, the physician hugs the patient in a comforting way. Is this a boundary crossing, boundary violation, or within the boundaries of medical practice?
- a. boundary crossing
  - b. boundary violation
  - c. within the boundaries of medical practice
19. A family physician has been seeing a patient for several months, and during a visit, the physician makes a sexual advance towards the patient. Is this a boundary crossing, boundary violation, or within the boundaries of medical practice?
- a. boundary crossing
  - b. boundary violation
  - c. within the boundaries of medical practice
20. Which of the following is not a common symptom of delirium?
- a. Confusion
  - b. Disorientation
  - c. Agitation
  - d. Difficulty in finding words (aphasia)
  - e. Euphoria
21. A patient with delirium is displaying symptoms of agitation and aggression. Which of the following drug classes would be most appropriate for managing these symptoms?
- a. Antidepressants
  - b. Antipsychotics

- c. Benzodiazepines
  - d. Anticonvulsants
22. Which of the following NOT one of the symptoms that classically presents with Alzheimer's disease?
- A. Anterograde amnesia
  - B. Anxiety
  - C. Apraxia
  - D. Executive dysfunction
  - E. Agnosia
23. Which one of the statements about Lewy Body Dementia is FALSE?
- A. Mean age of presentation is 75
  - B. Affected individuals commonly have no family history
  - C. Motor symptoms often present before cognitive symptoms
  - D. Patients can experience REM sleep disorders, repeated falls, and autonomic instability
24. What branch of psychiatry treats paraphilic disorders?
- A. Child Psychiatry
  - B. General Psychiatry
  - C. Liaison Psychiatry
  - D. Forensic Psychiatry
25. Which of the following is NOT a treatment option for paraphilic disorders?
- A. SNRIs – Duloxetine, Venlafaxine
  - B. Anti-androgen medication – cyproterone acetate
  - C. Luteinising hormone-releasing hormone (LHRH) agonists – Triptorelin, Goserelin
  - D. Psychotherapy – Cognitive behaviour interventions such as social skills training, cognitive restructuring, etc.
26. Which of the following are considered to be neuropsychiatric symptoms of dementia?
- A. Depression
  - B. Irritability

- C. Suspiciousness
- D. Hallucinations
- E. All of the above

27. Which of the following statements about treatment of neuropsychiatric symptoms of dementia is false?

- A. Non-pharmacologic treatments such as sensory enhancement, environmental interventions, and behaviour show limited effectiveness for treating aggression and psychosis
- B. Atypical antipsychotic medications are first line treatment for severe behavioral symptoms with psychotic features
- C. Cognitive enhancing medication such as Donepezil and Memantine have shown weak evidence for treatment of neuropsychiatric symptoms
- D. Atypical anti-psychotics are considered to be first line therapy for treatment of neuropsychiatric symptoms without psychotic features

## Answers

1. A2, B3, C4, D1
2. A:
3. A,C; Depression shows shortened REM latency, **increased** REM density, reduced slow wave sleep and **more** fragmentation. Therefore, B and D are incorrect.
4. D; Insomnia is not a clinical indication for a sleep study. It is a clinical diagnosis. Obstructive sleep apnea, narcolepsy and parasomnia are clinical indications for sleep studies.
5. B: Obstructive sleep apnea is a common cause of daytime sleepiness. Caused by the obstruction of the upper airway during sleep, leading to desaturations and transient arousals. Common symptoms also include high blood pressure, impotence and lack of sex drive, increased traffic and workplace accidents, cardiac problems, memory problems and inability to think correctly and increased insulin resistance.
6. C; For the classification of OSA, AHI for mild = 5-15, moderate = 15-30 and severe > 30hr
7. A,B,C; BiPAP is not indicated in OSA
8. A; CBT is first line therapy for insomnia with equivalent efficacy compared to medication but with less side effects and longer duration.
9. C; REM behaviour disorder is now known as a prosomal syndrome of alpha synuclein neurodegeneration such as Parkinson's disease, Multisystem Atrophy or Lewy Body Dementia. The rest of the options are related to obstructive sleep apnea.
10. A; The main difference between Type I and II Narcolepsy is the presence of Cataplexy. Excessive daytime sleepiness and SOREMPs are present in both types of Narcolepsy. **CSF Orexin-1 <100pg/ml and >100pg/ml for Type I and II Narcolepsy, not 1000pg/ml.**

11. C,D; Treatment for restless leg syndrome include iron supplementation, alpha-2-delta calcium channel ligands, and dopamine agonists.
12. D; She has a high level of anxiety about her health and a preoccupation with having or acquiring a serious illness. She has an illness preoccupation that has been present for at least 5 months. No somatic symptoms are present. All of which are characteristics of illness anxiety disorder.
13. B: Conversion Disorder is characterized by at least 1 symptom of altered voluntary motor or sensory function for at least 6 months with clinical findings that are incompatible with recognized neurological or medical conditions.
14. C. Medical and psychiatric disorders that have premenstrual exacerbation will continue to have symptoms continuously and will only worsen during the premenstrual period. Whereas PMDD will have severe symptoms in the final week before onset of menses, start to improve after onset of menses and will become minimal/absent in the week after menses.
15. B. SSRIs and TCAs do not have evidence of teratogenicity while bupropion use during early pregnancy is associated with increased risk of cardiac malformations.
16. A - II; B - I; C - III; D - IV.
17. D. Individuals with dual diagnosis have difficulties communicating and disorganized brain functions, often resulting in them expressing themselves behaviourally. They generally do not present with any growth abnormalities and so failure to thrive would not be a classic symptom.
18. A) Boundary crossing. A hug can be seen as a comforting gesture and can be beneficial for the patient, but it is not a typical part of the physician-patient relationship. It's crossing a boundary but not violative.
19. B) boundary violation. A physician should never make a sexual advance towards a patient as it is completely unacceptable and violative of the physician-patient relationship. It's violative as it's crossing a boundary that should not be crossed and it's a serious ethical breach. This kind of behavior is a violation of the trust and professionalism that is expected from a physician, and it's considered a form of sexual misconduct. It can also lead to legal and professional repercussions for the physician.
20. E) Euphoria. Euphoria is a feeling of intense joy or happiness and it's not a common symptom of delirium. Delirium is characterized by a severe disturbance in consciousness, attention, and perception. Common symptoms include confusion, disorientation, and agitation, as well as hallucinations and difficulty sleeping. Delirium is not a diagnosis of a primary emotional state, but rather a state of a severe disturbance in the brain function, often caused by an underlying medical condition or medication. Identifying and treating the underlying cause is the first step in managing a patient with delirium.
21. B) Antipsychotics are the treatment of choice for the symptoms of hyperactive delirium. The exceptions are if the patient is in alcohol withdrawal (in which case use benzodiazepine), or they are experiencing an anticholinergic toxidrome (in which case use physostigmine). Common medications are Haloperidol, Loxapine, Risperidone, Quetiapine, and Olanzapine. Make sure to monitor for QT prolongations.
22. B. Presentation of AD is described by the 4As and one D. Anterograde amnesia, Apraxia, Agnosia, Aphasia, and executive Dysfunction.

23. C. Motor symptoms often present AFTER cognitive symptoms. This is a defining feature of this condition that differentiates it from Parkinson's Disease Dementia, which has motor symptoms present before cognitive symptoms.
24. D
25. A. Tx for paraphilias includes SSRIs to reduce libido, anti-androgenic meds, LHRH agonists, and psychotherapy. Not SNRIs.
26. E
27. C. Cognitive enhancing medication have not shown any evidence at all for the treatment of neuropsychiatric symptoms. All the rest of the statements are true.